

Tue, Jan 13 at 11:12 PM What

kind of information do i need to gathering to write NDIS Assistive technology report after 2 month trial of bioness for MS patient?

Great question — a strong NDIS Assistive Technology (AT) report after a 2-month Bioness trial for a client with Multiple Sclerosis needs to clearly show need, evidence of benefit, and value for money.

Here's exactly what information you should gather and include:

1. Participant & Context NDIS

number

Diagnosis: Multiple Sclerosis (type if known)

Functional impacts (mobility, fatigue, balance, falls risk)

Living situation (home layout, community access needs)

Current mobility aids (AFO, cane, walker, wheelchair, etc.)

2. Baseline (Before Trial)

You must show what life was like before Bioness.

Mobility & Function

Walking distance (e.g. 20m, 100m, community only)

Gait pattern: foot drop severity, circumduction, hip hiking Speed (10MWT if available)

Endurance (6MWT if available)

Falls history (frequency, near-falls)

Fatigue level (subjective scale)

Participation

Ability to:

Shop independently Walk outdoors

Attend work / appointments Manage stairs or uneven surfaces

3. Trial Details

Document the structure of the trial.

Dates of trial (start–end)

Bioness model used

Wearing schedule (hours/day, days/week)

Training provided (who, when)

Support required (carer set-up vs independent)

Any issues: skin, tolerance, spasticity, tech problems

4. Outcomes

After 2 Months

This is the most important section.

Objective Changes (if available)

Walking speed improvement

Increased distance

Reduced tripping

Improved foot clearance

Better symmetry

Reduced reliance on walking aids

Machine Translated by Google

Subjective Changes

From participant, carer, and clinician:

Confidence walking

Reduced fear of falling

Fatigue change

Pain change

Willingness to go out more Quality
of life impact 5.

Comparison to Other Options

NDIS needs to see why Bioness is reasonable and necessary.

Compare against:

FIRE

No device

Physiotherapy alone

Include:

Why AFO was insufficient Why

Bioness is superior for THIS client Long-term

benefits vs short-term therapy only 6. Risk & Safety Falls
risk with and without
device

Safety on stairs, ramps, community terrain Risk of
deconditioning if not approved Carer safety
(less physical support needed)

7. Independence & Participation Outcomes

Link directly to NDIS goals:

Increased community access Return
to work/study Reduced
carer support Increased self-
management Maintaining function
as MS progresses 8. Cost & Value for Money

Include:

Cost of Bioness system

Expected lifespan

Cost comparison:

Ongoing physio

Falls injuries

Increased support hours Show
long-term savings 9. Clinical

Recommendation

Clear statement:

Device is reasonable and necessary. Not
luxury, not convenience — essential mobility support. Consequences if
not funded 10. Supporting Evidence
to Attach Trial outcome summary Gait
assessment

Physio letter
Falls record
Photos/video (if allowed)
Participant testimonial

If you'd like, I can now:

Give you a ready-to-use AT report template, or Draft
a sample completed report based on a typical MS + Bioness trial scenario.

Great — here's a ready-to-use NDIS Assistive Technology Report template specifically for a 2-month Bioness trial for a
participant with Multiple Sclerosis.

You can copy/paste this into Word and fill in each section.

NDIS Assistive Technology Report

Bioness L300 / L300 Go Trial – Multiple Sclerosis

1. Participant Details

Name:

NDIS Number:

Date of Birth:

Primary Diagnosis: Multiple Sclerosis (type if known)

Referring Clinician:

Report Author:

Date of Report:

2. Purpose of Report

This report supports an application for funding of the Bioness Functional Electrical Stimulation (FES) system following a 2-
month trial, demonstrating its impact on the participant's mobility, safety, and functional independence.

3. Participant Background

Living situation:

Daily mobility demands (home / community / work):

Current supports (carer, physio, OT):

Current mobility aids (AFO, cane, walker, wheelchair):

Relevant medical considerations (fatigue, spasticity, sensation, skin integrity):

4. Baseline Function (Pre-Trial)

Mobility

Walking distance tolerance:

Gait pattern:

Foot drop severity:

Falls history (last 6–12 months):

Walking aids required:

Functional Impact

Ability to access community:

Ability to complete daily tasks independently:

Confidence with mobility:

5. Trial Overview

Device trialled: Bioness (model)

Trial period: (dates)

Set-up & training provided by:

Wearing schedule:

Support required for donning/doffing:

Any issues during trial:

Skin

Tolerance

Spasticity

Technical difficulties

6. Outcomes After 2 Months

Objective Outcomes

Walking speed change:

Walking distance change:

Foot clearance improvement:

Reduced tripping / stumbling:

Reduced reliance on walking aids:

Subjective Outcomes

Participant report:

Confidence:

Fatigue:

Pain/discomfort:

Willingness to engage in community activities:

Carer / family feedback:

Safety improvements:

Reduced physical support required:

Clinician observations:

Gait efficiency:

Endurance:

Balance:

7. Comparison to Other Interventions AFO

Limitations experienced:

Why AFO alone is insufficient:

Physiotherapy Alone

Benefits but limitations:

Why device support is still required: Why

Bioness Is Most Appropriate Dynamic

assistance vs passive support Adaptability to MS

progression Functional carryover into

daily life 8. Risk & Safety Considerations

Falls risk without Bioness:

Falls risk with Bioness:

Safety on stairs, ramps, uneven surfaces:

safety considerations: 9. Impact on

Independence & NDIS Goals

The Bioness system supports the participant to:

Increase community access

Maintain employment / study participation

Reduce reliance on carers

Improve self-management of mobility

Maintain function despite disease progression

(Align this directly with stated NDIS goals.)

10. Cost & Value for Money Cost

of device: \$ Expected

lifespan: Ongoing

costs: (consumables, servicing)

Cost Avoidance

Reduced therapy reliance

Reduced falls-related injury risk

Reduced support hours

Delayed need for higher-level mobility aids

11. Clinical Recommendation

Based on the 2-month trial outcomes, the Bioness system is strongly recommended as a reasonable and necessary assistive technology support.

The device:

Significantly improves gait safety and efficiency

Reduces falls risk

Increases independence and participation

Represents value for money compared to ongoing high-cost supports

Without this device, the participant is at high risk of:

Reduced mobility

Increased falls

Increased reliance on carers

Reduced community participation 12.

Attachments

Trial summary

Gait assessment

Physiotherapy letter

Falls record

Participant statement

Photos/videos (if appropriate)

If you'd like, I can now:

Turn this into a Word document for you, or Draft a

completed sample report using a typical MS + Bioness trial case.

Draft a completed sample report

You can copy this directly into Word and edit names/dates as needed.

NDIS Assistive Technology Report

Bioness L300 Go Trial – Multiple Sclerosis

1. Participant Details

Name: Jane Smith

NDIS Number: 4312 567 890

Date of Birth: 14 March 1989

Primary Diagnosis: Relapsing-Remitting Multiple Sclerosis

Referring Clinician: Dr Michael Brown, Neurologist

Report Author: Mee Young Lee, Orthotist

Date of Report: 13 January 2026

2. Purpose of Report

This report is submitted to support an application for NDIS funding for a Bioness L300 Go Functional Electrical Stimulation (FES) system following a two-month trial period. The trial aimed to evaluate the impact of the device on the participant's gait efficiency, safety, independence, and community participation.

3. Participant Background

Jane lives independently in a single-storey home and works part-time as an administrative assistant. She experiences progressive right-sided weakness associated with MS, resulting in significant foot drop, reduced walking endurance, and frequent near-falls.

She currently receives:

Physiotherapy: once weekly

OT support: monthly

No formal carer support

Current mobility aids include a carbon fibre AFO and a single-point cane for community ambulation.

Primary functional concerns include:

High fatigue levels after short walking distances

Reduced confidence in outdoor mobility

Fear of falling, particularly on uneven surfaces

Reduced community engagement

4. Baseline Function (Pre-Trial)

Mobility

Walking distance: ~60–80 m before requiring rest Gait pattern:

Right foot drop with circumduction and hip hiking Walking aids: AFO + cane

outdoors Falls history: 3 falls and multiple

near-falls in the previous 6 months 10-Metre Walk Test: 0.68 m/s

Functional Impact

Jane reported limiting community outings due to fear of tripping and fatigue. She avoided uneven ground and required frequent rest breaks when shopping or attending appointments. Confidence with mobility was low, particularly when fatigued.

Trial period: 10 November 2025 – 10 January 2026

Set-up & training: Provided by orthotist with physiotherapy input

Wearing schedule:

Week 1–2: 1–2 hours/day

Week 3 onwards: up to 6 hours/day in community and work settings

Independence: Jane was able to don/doff the device independently after 1 week

Issues during trial:

Mild skin redness initially, resolved with strap adjustment

No adverse effects or intolerance

6. Outcomes After 2 Months

Objective Outcomes

Walking speed: Improved from 0.68 m/s to 0.86 m/s Walking

distance: Increased from ~80 m to over 200 m without rest Gait quality: Improved foot clearance

Reduced circumduction

Improved step symmetry

Falls: No falls during the 2-month trial period

Walking aids: Reduced reliance on cane in familiar indoor environments

Subjective Outcomes

Participant report:

“I feel much safer walking now. I don’t have to constantly think about lifting my foot. I’m less tired at the end of the day and I’ve started going out more again.”

Jane reported:

Increased confidence in outdoor walking

Reduced fear of falling

Improved endurance

Greater willingness to attend social and community activities

Physiotherapist feedback:

Improved gait efficiency and reduced compensatory movements Improved

dynamic balance during walking Increased tolerance

for therapy sessions due to reduced fatigue 7. Comparison to Other Interventions

AFO

While Jane’s AFO provides passive ankle support, it:

Does not address fatigue-related foot drop progression

Restricts ankle movement and limits dynamic gait

Has contributed to increased compensatory movements at the hip

Physiotherapy Alone

Physiotherapy has improved strength and balance but has not been sufficient to manage the neurological component of foot drop, particularly as fatigue increases throughout the day.

Why Bioness Is Most Appropriate

The Bioness system provides active, timed dorsiflexion, supporting a more natural gait pattern and reducing compensatory strategies. It adapts well to day-to-day variability in MS symptoms and enables functional carryover into daily life.

8. Risk & Safety Considerations

Without Bioness:

- High risk of falls due to inconsistent foot clearance
- Increased fatigue leading to gait deterioration
- Reduced community participation and physical deconditioning

With Bioness:

- Improved stability on stairs, ramps, and uneven surfaces
- Reduced falls risk
- Improved confidence and safety in community ambulation
- Reduced reliance on others for supervision

9. Impact on Independence & NDIS Goals

The Bioness system directly supports Jane's NDIS goals to:

- Maintain independence in daily mobility
- Increase community participation
- Sustain employment
- Reduce reliance on informal supports
- Maintain physical function despite progressive neurological changes

Since commencing the trial, Jane has increased her weekly community outings and resumed independent grocery shopping without assistance.

10. Cost & Value for Money

- Cost of Bioness L300 Go: \$9,500 (estimate)
- Expected lifespan: 4–5 years
- Ongoing costs: Replacement electrodes and servicing
- Value for Money

Funding this device is expected to:

- Reduce the need for increased physiotherapy input
- Reduce falls-related injury risk and associated medical costs
- Delay progression to higher-level mobility aids
- Reduce future need for support worker assistance

Over the lifespan of the device, this represents significant cost avoidance compared to ongoing support hours and injury management.

11. Clinical Recommendation

Based on the outcomes of the two-month trial, it is strongly recommended that the NDIS approve funding for the Bioness L300 Go system for Jane Smith.

The device has demonstrated clear benefits in:

- Improving gait safety and efficiency
- Reducing falls risk

Increased independence and community participation
Supporting long-term functional mobility in the context of progressive MS

Without this device, Jane is at increased risk of:

Reduced mobility and confidence
Increased falls and injury
Greater reliance on formal and informal supports
Reduced quality of life and participation

The Bioness system therefore meets the NDIS criteria of being reasonable and necessary.

12. Attachments

Physiotherapy outcome summary
Gait assessment results
Participant statement
Falls record (pre-trial)

If you'd like, I can now:

Convert this into a Word document, or
Customise it using your real client details so it's ready for submission.
NDIS AT templet for ORTHOTICS

Here's a professional, ready-to-use NDIS Assistive Technology (AT) Report Template for ORTHOTICS.
This is suitable for AFOs, KAFOs, SMOs, footwear modifications, spinal orthoses, upper-limb splints, etc.

You can copy this straight into Word and use it for submissions.

NDIS Assistive Technology Report – Orthotics

1. Participant Details

Name:
NDIS Number:
Date of Birth:
Primary Diagnosis:
Secondary Conditions (if relevant):
Address / Living Situation:
NDIS Goals (relevant excerpts):

2. Report Details

Referring Practitioner:
Report Author: (Name, title – Orthotist)
Organisation:
Date of Report:

3. Purpose of Report

This report supports an application for NDIS funding for custom orthotic intervention to address the participant's functional limitations, improve safety and independence, and support achievement of NDIS goals.

4. Participant Background

Describe:

Daily mobility demands (home, community, work/school)
Current supports (physio, OT, carers)
Current assistive devices (if any)

Spasticity
Muscle weakness
Sensory impairment
Fatigue
Skin integrity
Contractures

5. Current Functional Status (Without Proposed Orthosis)

Mobility & Posture Gait

pattern:

Standing balance:

Transfers:

Endurance:

Falls / near-falls history:

Functional Impact

Impact on daily activities: Impact

on community access: Impact on

work/school participation: Carer assistance

required: 6. Previous / Current

Interventions

Previous orthoses used:

Physiotherapy input:

OT input:

Medical interventions:

Outcomes and limitations of previous supports: 7.

Assessment Findings

(Include clinical reasoning)

Joint range of motion:

Muscle strength:

Tone/spasticity:

Sensation:

Skin condition:

Postural alignment:

Functional gait assessment: 8.

Recommended Orthotic Intervention

Device Details

Type of orthosis: (e.g., Custom hinged AFO, solid AFO, KAFO, TLSO, wrist splint)

Laterality: (Left / Right / Bilateral)

Design features:

Materials

Joints

Straps

Accommodations

Footwear requirements (if applicable): 9. Clinical

Rationale

Explain why this device is required:

The recommended orthosis is required to:

Improve joint alignment

Improve gait efficiency

Reduce falls risk

Reduce pain and fatigue

Prevent secondary complications (contracture, deformity)

Support functional independence 10.

Expected Outcomes

With the orthosis, the participant is expected to:

Walk more safely and efficiently

Increase endurance

Reduce reliance on carers

Improve confidence in mobility

Maintain participation in daily and community activities

Prevent deterioration of function

11. Comparison to Alternative Options No

Device

Risks and limitations:

Off-the-shelf / Lower-cost Option

Why insufficient:

Physiotherapy Alone

Benefits but limitations:

Why Custom Orthotics Is Most Appropriate

Individualised fit

Long-term durability

Clinical necessity 12.

Risk & Safety Considerations Without

Orthosis

Falls risk

Joint instability

Pain

Reduced participation

With Orthosis

Improved safety

Improved postural support

Reduced injury risk

Increased independence 13.

Impact on NDIS Goals

The recommended orthotic intervention directly supports the participant's goals to:

Increase independence

Improve mobility

Participate in community life

Maintain employment/education

Reduce reliance on supports 14.

Cost & Value for Money Cost of
orthosis: \$

Expected lifespan:

Review/maintenance plan:

Value for Money

This intervention represents value for money by:

Machine Translated by Google

Reducing falls-related injury risk Reducing
future therapy and medical costs Delaying need for
higher-level mobility aids Reducing reliance on support
workers 15. Training & Implementation Fitting
and adjustment plan: Wearing-in
schedule: Education provided to
participant/carer: Follow-up
schedule: 16. Clinical Recommendation

Based on the assessment findings, it is strongly recommended that the NDIS approve funding for the proposed custom orthotic intervention.

This device is:

Clinically necessary
Directly linked to functional outcomes
Reasonable and necessary under NDIS criteria
Essential to maintain safety, independence, and participation

Without this intervention, the participant is at high risk of:

Increased falls
Reduced mobility
Increased dependence on supports Reduced
quality of life 17. Attachments

Clinical assessment notes
Gait analysis
Physiotherapy/OT reports
Photos (if appropriate)
Participant statement

If you'd like, I can now: ÿ

Turn this into a Word document ÿ Create a
short-form version for low-cost AT ÿ Or customise it for a
specific device (AFO, KAFO, TLSO, wrist splint, footwear mods, etc.)